# Case study: Daniel

Condition: **Stroke – left middle cerebral artery ischaemic stroke**

46-year-old man · Inpatient rehabilitation unit, public hospital

Daniel Okafor is a 46-year-old man who is an inpatient on a rehabilitation unit following a **left middle cerebral artery ischaemic stroke** three weeks ago. Daniel works full-time as an IT project manager and lives with his wife, Grace, and their two teenage children. Since his stroke, Daniel has had weakness in his right arm and mild difficulty finding words, although he understands conversation well. His main goal is to return to work early next year, and he often talks about the projects waiting for him.

Daniel has been progressing steadily in rehabilitation. His physiotherapist reports that he is walking independently around the unit, and his speech pathologist has noted that his word-finding has improved over the past week. He is eating a normal diet and drinking thin fluids. Earlier this morning, a nurse mentioned in passing that Daniel's blood pressure had been "up and down" over the last few days. Yesterday, Grace told you she is worried about the family's income and asked whether you could help her complete forms for Daniel's income protection insurance.

You have been asked to run a kitchen session with Daniel at 10:15 am, in which he will make a toasted sandwich. He arrives on time and tells you he is tired after a bad night, but he is keen to practise. His speech is at its usual level, with occasional pauses to find words. He collects the bread, cheese and tomato from the fridge without prompting, and butters the bread, using his right hand to steady it as you have practised together. His phone, on the bench, buzzes repeatedly with work emails, and he jokes, "They can't cope without me."

At about 10:35 am, while cutting the tomato, Daniel's speech suddenly becomes more slurred than it was at the start of the session. When he smiles, the right side of his face droops more obviously than it did earlier. He drops the knife and cannot pick it up again. When you ask him to squeeze your fingers, his right-hand grip is much weaker than when you started. Daniel tells you his head feels "funny" and rubs his forehead.

Daniel is embarrassed. "I'm just tired," he says. "Bad night. Let's keep going, I want to finish this." He glances at his phone and says he needs to reply to his manager before lunch. When you ask what he means by "funny", he struggles to find the words and says, "Just… not right."

There is no clear documentation in the file of Daniel's most recent blood pressure or blood glucose readings, or of what the nurse meant by "up and down". You do not know whether Daniel had any similar symptoms during his bad night, whether anything was reported to nursing staff overnight, or which medications he is currently taking.

It is now 10:40 am. Daniel reaches for the glass of water on the bench, saying he just needs a drink and something to eat and then he will finish the sandwich. At the same moment, his physiotherapist knocks on the kitchen door. Daniel's physiotherapy session is due to start in 20 minutes, and she asks whether she can take him down to the gym early because her afternoon is fully booked. Daniel starts to push himself up from the chair, saying, "Yes, let's go."

**You are the occupational therapy student running Daniel's kitchen session and your clinical supervisor is not currently in the room.**

**Present Daniel's case to your clinical supervisor using the ISBAR framework. Identify the immediate concerns and explain what you would prioritise in responding to the situation.**

# ISBAR planning sheet

Use your 10 minutes of preparation to plan. Then speak for 7 minutes to your “supervisor”.

## I Introduction

*≈ 20 s*

Who you are, your role, who the client is and why you need to talk now.

## S Situation

*≈ 40 s*

A brief but complete summary of what is happening and the immediate concern.

## B Background

*≈ 1 min*

Relevant history: the primary problem, medical and social history, medications/allergies, interventions so far and OT to date. Name missing information rather than assuming it.

## A Assessment

*≈ 2 min 20 s*

Your interpretation. Case finding → what it may mean → consequence for safety and occupations → priority. Separate facts, interpretation, gaps and proposed assessment. What needs escalation?

## R Recommendation

*≈ 2 min 40 s*

What should happen next, in priority order. Action → reason → who → review. Immediate safety, escalation, MDT input, one short-term and one longer-term OT assessment, intervention, referrals and follow-up.

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